

MERCY GENERAL HOSPITAL

OPERATIVE REPORT

Department of Neurosurgery

Patient Name:	MARTINEZ, SARAH
Date of Birth:	03/15/1987
MRN:	2847592
Date of Surgery:	09/18/2025
Surgeon:	Dr. Richard Stevens, MD, FAANS
Assistant Surgeon:	Dr. Emily Rodriguez, MD
Anesthesiologist:	Dr. James Wilson, MD
Circulating Nurse:	Maria Gonzalez, RN
Scrub Tech:	David Lee, CST

PREOPERATIVE DIAGNOSIS:

C5-C6 disc herniation with spinal cord compression and right C6 radiculopathy secondary to motor vehicle collision

POSTOPERATIVE DIAGNOSIS:

Same as preoperative diagnosis

PROCEDURE PERFORMED:

Anterior cervical discectomy and fusion (ACDF) at C5-C6 with insertion of interbody spacer and anterior cervical plate fixation

ANESTHESIA:

General endotracheal anesthesia

ESTIMATED BLOOD LOSS:

Approximately 75 mL

COMPLICATIONS:

None

SPECIMENS:

C5-C6 herniated disc material sent to pathology

IMPLANTS/DEVICES:

1. PEEK interbody spacer, 6mm height, filled with demineralized bone matrix
2. Anterior cervical plate with four 14mm titanium screws (Medtronic cervical plate system)

FLUOROSCOPY TIME:

2 minutes 45 seconds

OPERATIVE TIME:

Start: 08:15, Finish: 10:42 (Total: 2 hours 27 minutes)

INDICATIONS FOR SURGERY:

This is a 38-year-old female who sustained injuries in a motor vehicle collision on 08/12/2025. She presented with persistent neck pain, right upper extremity radiculopathy with weakness, and paresthesias in the right C6 distribution. MRI of the cervical spine demonstrated a large C5-C6 disc herniation with central and right paracentral components causing moderate central canal stenosis with spinal cord compression and moderate right neural foraminal stenosis with impingement on the right C6 nerve root. Conservative management including physical therapy, anti-inflammatory medications, and muscle relaxants failed to provide adequate relief. Given the severity of spinal cord compression, neurological deficits, and failure of conservative treatment, the patient was offered surgical decompression. The risks, benefits, and alternatives to surgery were discussed in detail with the patient, including risks of infection, bleeding, nerve injury, dural tear, cerebrospinal fluid leak, hardware failure, nonunion, adjacent segment degeneration, dysphagia, hoarseness, Horner's syndrome, stroke, and death. The patient understood and consented to proceed with surgery.

DESCRIPTION OF PROCEDURE:

The patient was brought to the operating room and identified by name, medical record number, and date of birth. The surgical site was verified and marked. Time-out was performed confirming correct patient, procedure, and surgical site. The patient was positioned supine on the operating table. General endotracheal anesthesia was induced without complications. A shoulder roll was placed to extend the neck. The patient's head was positioned in a neutral position and secured in a foam headrest. Both arms were tucked at the sides and well-padded. Pressure points were carefully padded. Sequential compression devices were applied to both lower extremities for DVT prophylaxis.

The anterior neck was prepped and draped in the usual sterile fashion with chlorhexidine gluconate solution. Fluoroscopy was used to identify the C5-C6 disc space. A horizontal incision approximately 4cm in length was marked on the right side of the neck at the level of C5-C6, along the anterior border of the sternocleidomastoid muscle. The skin was infiltrated with 0.5% lidocaine with epinephrine 1:200,000 for hemostasis.

The skin incision was made with a #15 blade scalpel. Hemostasis was achieved with bipolar electrocautery. The platysma muscle was identified and divided in line with the skin incision. Subplatysmal flaps were raised superiorly and inferiorly. The anterior border of the sternocleidomastoid muscle was identified. Blunt dissection was carried down medial to the carotid sheath and lateral to the trachea and esophagus. The prevertebral fascia was identified and incised. Self-retaining Caspar retractors were placed under the longus colli muscles bilaterally for exposure.

Intraoperative fluoroscopy confirmed the correct level at C5-C6. A needle was placed into the disc space for confirmation. Attention was then turned to the discectomy. The anterior longitudinal ligament was incised sharply. A complete C5-C6 discectomy was performed using a combination of pituitary rongeurs, curettes, and angled Kerrison rongeurs. The disc space was carefully cleaned of all disc material and cartilaginous endplates down to bleeding subchondral bone. The posterior longitudinal ligament was identified and carefully removed to decompress the spinal cord and nerve roots.

Upon removal of the herniated disc material and posterior longitudinal ligament, there was excellent decompression of the thecal sac and right C6 nerve root. The dura was noted to be pulsatile with good CSF flow. No dural tears were identified. Hemostasis was achieved with bipolar electrocautery and thrombin-soaked Gelfoam.

The disc space was then measured and prepared for fusion. A 6mm PEEK interbody spacer filled with demineralized bone matrix was selected. Trial spacers were used to confirm appropriate sizing. The interbody spacer was carefully impacted into the disc space under fluoroscopic guidance, achieving good position with restoration of disc height and lordosis. Fluoroscopy confirmed excellent position of the graft.

An appropriately sized Medtronic anterior cervical plate was then selected. The plate was positioned over the C5 and C6 vertebral bodies. Four self-drilling, self-tapping 14mm titanium screws were placed, two in C5 and two in C6, under fluoroscopic guidance. All screws had excellent purchase in

bone. The locking caps were engaged. Final fluoroscopic images confirmed excellent hardware position with appropriate alignment and no hardware violation of adjacent disc spaces or neural foramina.

The wound was copiously irrigated with antibiotic irrigation solution (bacitracin 50,000 units in 500mL normal saline). Hemostasis was excellent. A #10 flat Jackson-Pratt drain was placed in the prevertebral space and brought out through a separate stab incision. The platysma was reapproximated with 3-0 Vicryl interrupted sutures. The skin was closed with 4-0 Monocryl subcuticular sutures. Dermabond skin adhesive was applied. Sterile dressing was applied.

The patient tolerated the procedure well. There were no complications. All sponge, needle, and instrument counts were correct at the end of the procedure. The patient was extubated in the operating room and transferred to the post-anesthesia care unit in stable condition. Neurological examination immediately post-procedure showed intact motor and sensory function in all extremities with improved strength in the right upper extremity compared to preoperative examination.

POSTOPERATIVE PLAN:

1. Transfer to neurosurgery floor when stable
2. Pain control with oral analgesics
3. Remove drain when output less than 30mL per 8-hour shift
4. Soft cervical collar for comfort for 2 weeks
5. Early mobilization with physical therapy
6. Dysphagia precautions - start with clear liquids, advance as tolerated
7. Continue perioperative antibiotics for 24 hours
8. DVT prophylaxis with sequential compression devices and early mobilization
9. Anticipated discharge on postoperative day 1 or 2 if stable
10. Follow-up in clinic in 2 weeks with X-rays
11. No lifting greater than 5 pounds, no excessive neck rotation or extension for 6 weeks

PROGNOSIS:

Good. The patient underwent successful decompression of the spinal cord and C6 nerve root with fusion and stabilization of the C5-C6 segment. Immediate postoperative neurological examination showed improvement in right upper extremity strength. With appropriate postoperative care and physical therapy, the patient is expected to have good resolution of radicular symptoms and return to normal activities.

Electronically signed by:

Dr. Richard Stevens, MD, FAANS

Board Certified Neurosurgeon

NPI: 3456789012

Date Dictated: 09/18/2025

Date Transcribed: 09/18/2025

Date Signed: 09/19/2025